

# THE RESULTS OF YOUR ANALYSIS

PREPARED FOR

**Amanda**

DATE OF INJURY : N/A

DATE OF ANALYSIS : 7/31/2017

DATE OF IMAGES : 7/31/2017

REFERRING DOCTOR :

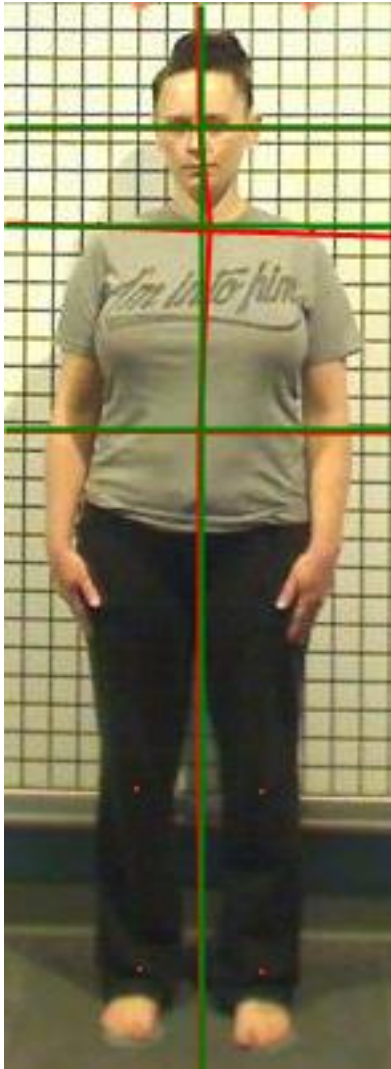
David Bohn

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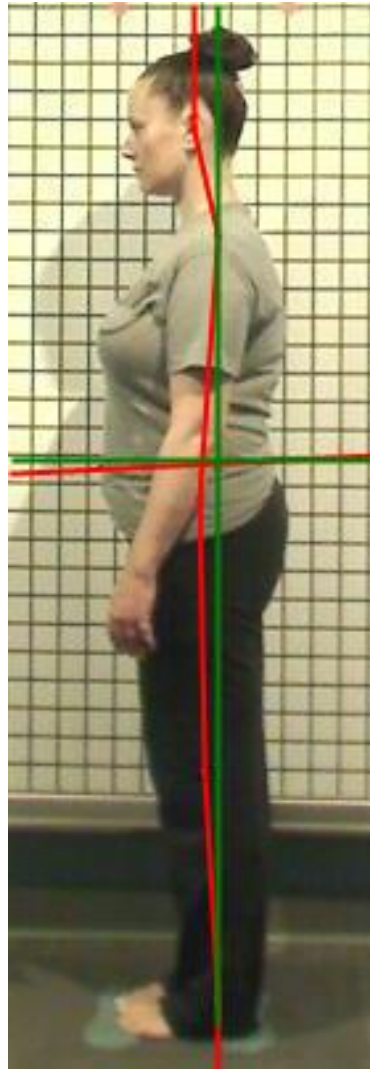
This report contains important information concerning structural changes that can be affecting your overall level of health and well-being. Spinal stability is a basic requirement for the protection of your nervous structures and the prevention of early mechanical deterioration of your spinal component

Instability is generally considered to be a global increase in the movements associated with the occurrence of back, neck, and/or nerve root pain. Damage to any spinal structure produces some degree of spinal instability.

This computer aided digital analysis is an overview of your current level of spinal stability.



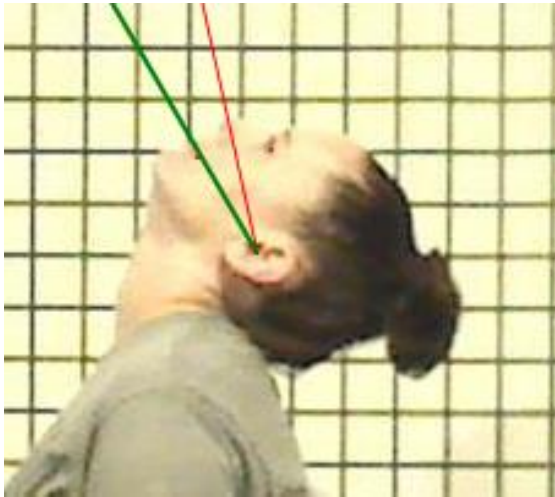
Head Tilt (inches) : 0  
Low Shoulder (inches) : 0.56  
Low Hip (inches) : 0.19  
Head Translation (inches) : 0.19  
Thoraic Translation (inches) : -0.56  
Pelvic Translation (inches) : 0.19  
Head Angle : 0  
Shoulder Angle : 1.96  
Pelvic Angle : 0.39



Body Weight(lbs) : 150  
AP Head Translation(inches) : 1.74  
Your head actually weighs : 12.34  
Due to the shift in posture your head feels like it weighs(lbs) : 33.81  
Shoulder Shift(inches) : 0  
Hip Shift(inches) : 1.35  
Hip Angle(deg) : 2.86

# Cervical Motion Analysis

Cervical Extension



Performed : 78 Degrees / Normal 60

0% Loss of Motion

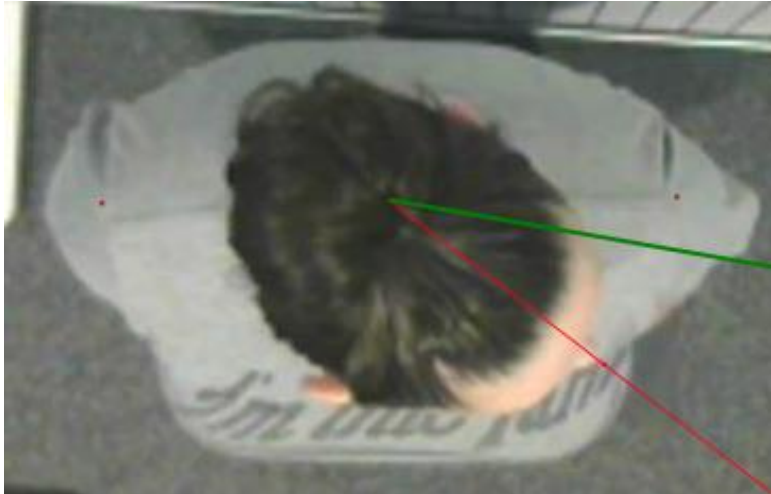
Cervical Flexion



Performed : 61 Degrees / Normal 50

0% Loss of Motion

Cervical Left Rotation



Performed : 53 Degrees / Normal 80

33.8% Loss of Motion

Cervical Right Rotation



Performed : 63 Degrees / Normal 80

21.2% Loss of Motion

Cervical Latflex Left



Performed : 47 Degrees / Normal 45

0% Loss of Motion

Cervical Latflex Right



Performed : 54 Degrees / Normal 45

0% Loss of Motion

# Lumbar Motion Analysis

Lumbar Flexion



Performed : 94 Degrees / Normal 60  
0% Loss of Motion

Lumbar Extension



Performed : 48 Degrees / Normal 25  
0% Loss of Motion

Lumbar Left APFlexion



Performed : 39 Degrees / Normal 25  
0% Loss of Motion

Lumbar Right APFlexion



Performed : 30 Degrees / Normal 25  
0% Loss of Motion



# Lateral Cervical Gravity Line

STANDARD RESULT



PATIENT RESULT



Result :  
Lateral Cervical Gravity Line : 6 mm Anterior

The Cervical Gravity Line is created by first locating the center of the tip of the Odontoid Process, then dropping a line downward, perpendicular to the bottom of the film. This line should just touch the anterior body of C7. When the line falls forward of the anterior body of C7 abnormal anterior head translation or forward head posture is present.

# Normal Lateral Cervical Curvature

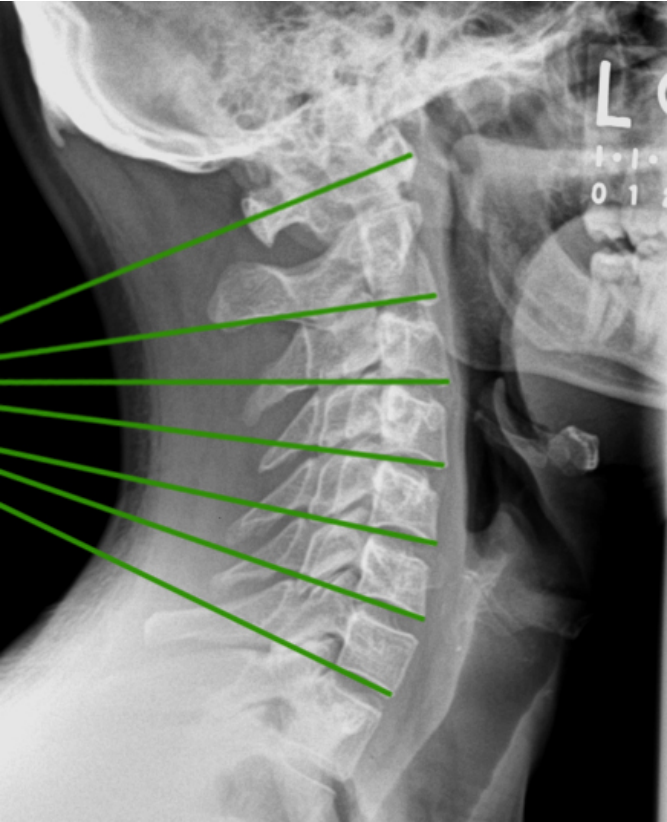
## PATIENT RESULT



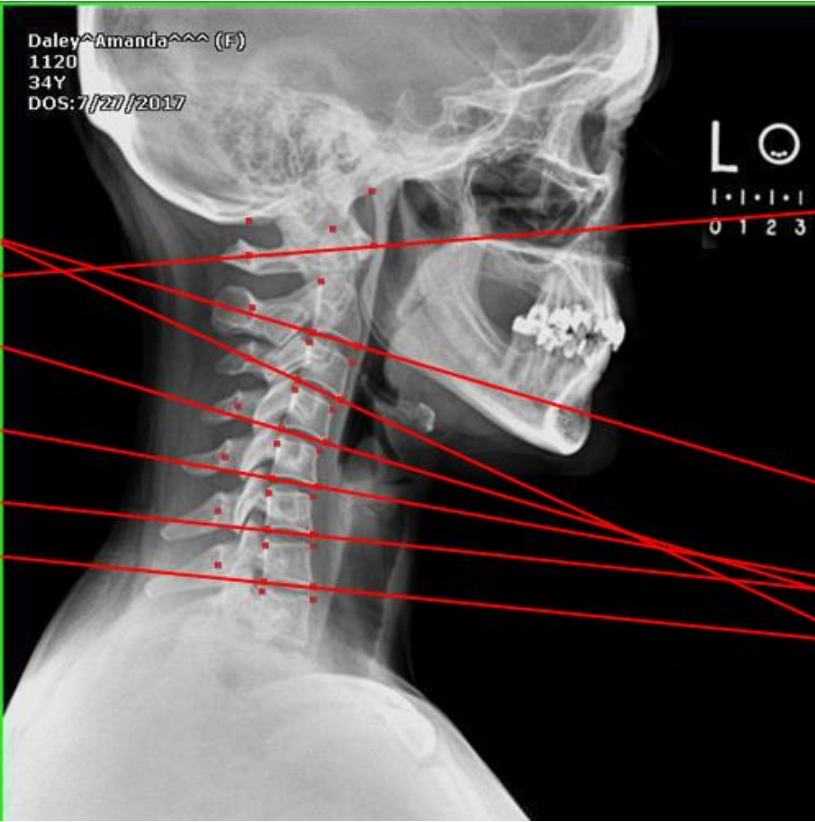
The image above shows your vertebral position in red and average or ideal in green. The red lines and the green line should be one on top of the other. Any deviation is generally considered to be abnormal.

# Lateral Cervical Baseline

STANDARD RESULT



PATIENT RESULT

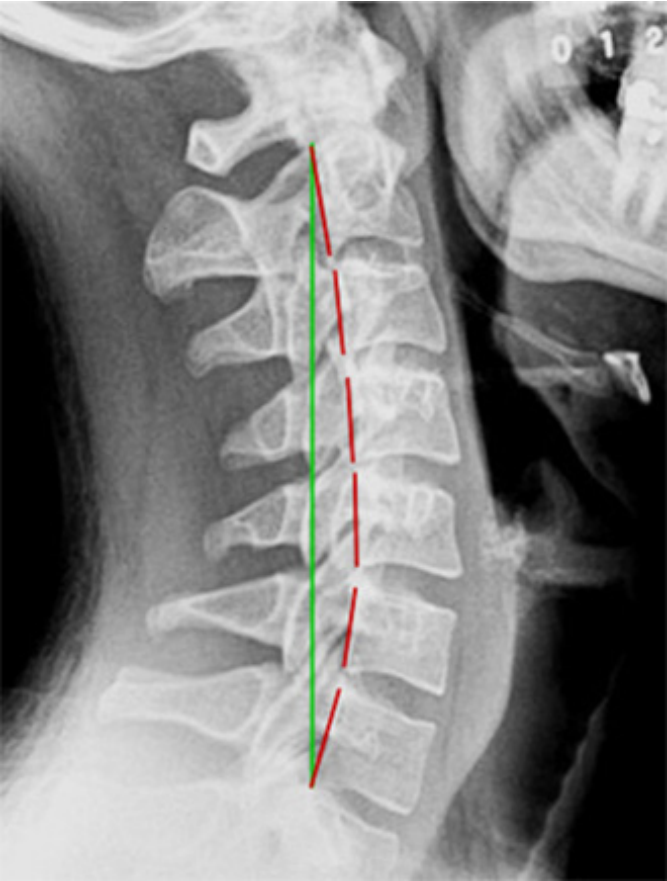


Result :  
Lateral Cervical Baseline : Abnormal

Lateral base lines extend posteriorly from the inferior epiphyseal plates of each lumbar vertebra. These lines normally meet posteriorly at a common point. If a base line intersects with its superior vertebra’s base line, fixed flexion of the inferior vertebrae is present. If a vertebrae’s base line intersects with its inferior vertebra, then fixed extension of the superior vertebrae is present. Fixed flexion or extension of a segmental unit can cause biomechanical dysfunction, which can lead to premature spinal degeneration.

# Forward Head Posture (FHP)

STANDARD RESULT



PATIENT RESULT



Result :  
Forward Head Posture FHP : 18.75 mm

One of the first indicators of poor posture is a slouching or forward head posture. This posture causes strain on the posterior neck muscles and increases loading on the spinal discs. For every inch (25.4 mm) of forward translation the weight-bearing load on the cervical spine increases by approximately 10 pounds. Car accidents, sports injuries, working with computers and loss of bone density can contribute to this abnormality. FHP creates muscle strain and uneven wearing of the discs and joints of the cervical spine. The abnormal spinal weight bearing associated with this type of posture can lead to premature spinal degeneration of the spinal joints and discs.



# Lateral Cervical Spondylolisthesis

STANDARD RESULT



PATIENT RESULT



Result :  
C2-C3 : 3.70 %  
C3-C4 : 0.65 %  
C4-C5 : 0 %  
C5-C6 : 2.62 %  
C6-C7 : 1.88 %

Spondylolisthesis describes the anterior displacement of a vertebra or the vertebral column in relation to the vertebrae below. The Meyerding grading system categorizes severity based upon measurements on lateral X-ray of the distance from the posterior edge of the superior vertebral body to the posterior edge of the adjacent inferior vertebral body. This is reported based on the total superior vertebral body length. Grade 1 is 0–25%, Grade 2 is 25–50%, Grade 3 is 50–75%, Grade 4 is 75–100%, and over 100% is Spondyloptosis, when the vertebra completely falls off the supporting vertebra.

# Cervical Spinal Stenosis

## PATIENT RESULT



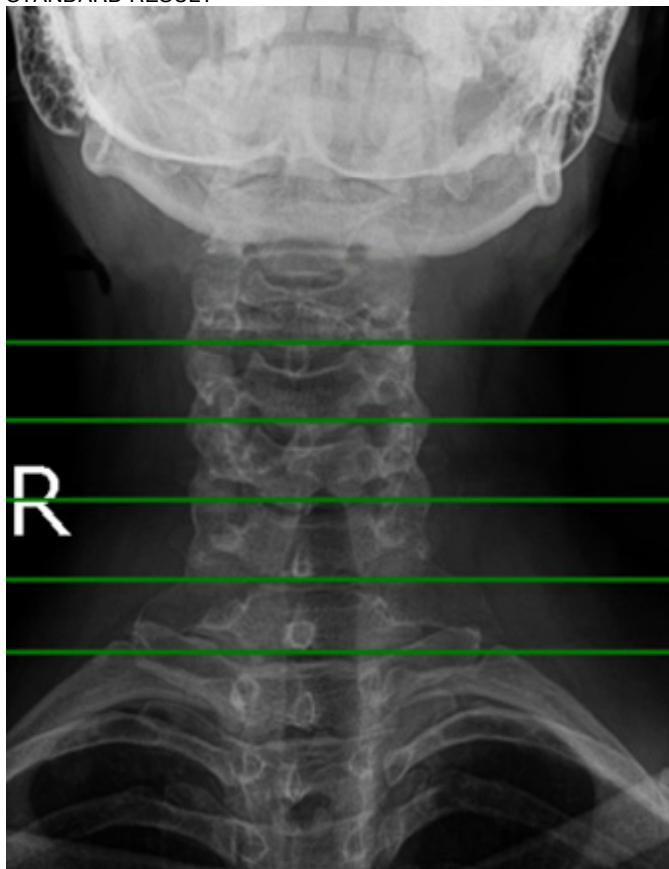
### Result :

- C2-C3 : 20.89 mm
- C3-C4 : 17.68 mm
- C4-C5 : 16.61 mm
- C5-C6 : 16.07 mm
- C6-C7 : 16.07 mm
- C7-C8 : 15 mm

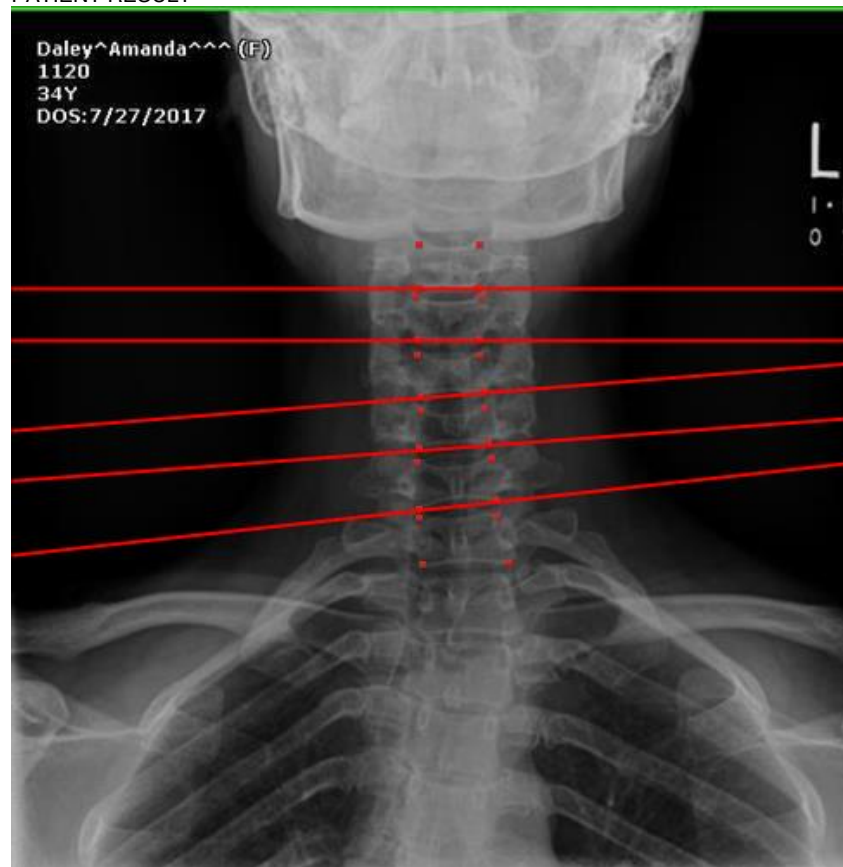
In adults, the width of the spinal canal on x-ray is normally 13 mm. Patients with spinal stenosis due to degenerative disease are at increased risk of spinal cord injury with minor trauma. As the diameter of the spinal canal decreases, pressure can increase on the spinal cord or the nerve roots. Pressure on these nerves in the spinal cord can cause numbness, tingling, or pain in the arms, hands, and legs. This condition is sometimes called cervical myelopathy.

# AP Cervical Baselines

STANDARD RESULT



PATIENT RESULT



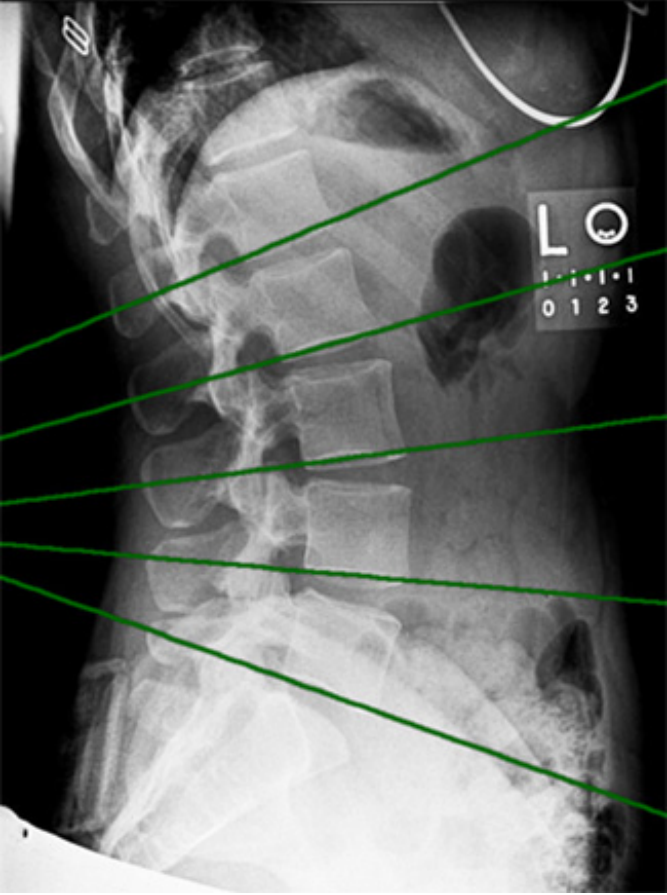
Result :

AP Cervical Baselines : Abnormal

Anterior-posterior baselines evaluate the spine for the presence of lateral flexion subluxations. Baselines are drawn across the inferior endplate of each cervical vertebra and should be parallel to a true horizontal line. Abnormal baselines can indicate the presence of dysfunction that may cause nerve irritation or accelerate degenerative spinal changes.

# Lateral Lumbar Baselines

STANDARD RESULT



PATIENT RESULT



Result :  
Lateral Lumbar Baselines : Abnormal

Lateral base lines extend posteriorly from the inferior epiphyseal plates of each lumbar vertebra. These lines normally meet posteriorly at a common point. If a base line intersects with its superior vertebra's base line, fixed flexion of the inferior vertebrae is present. If a vertebrae's base line intersects with its inferior vertebra, then fixed extension of the superior vertebrae is present. Fixed flexion or extension of a segmental unit can cause biomechanical dysfunction, which can lead to premature spinal degeneration.

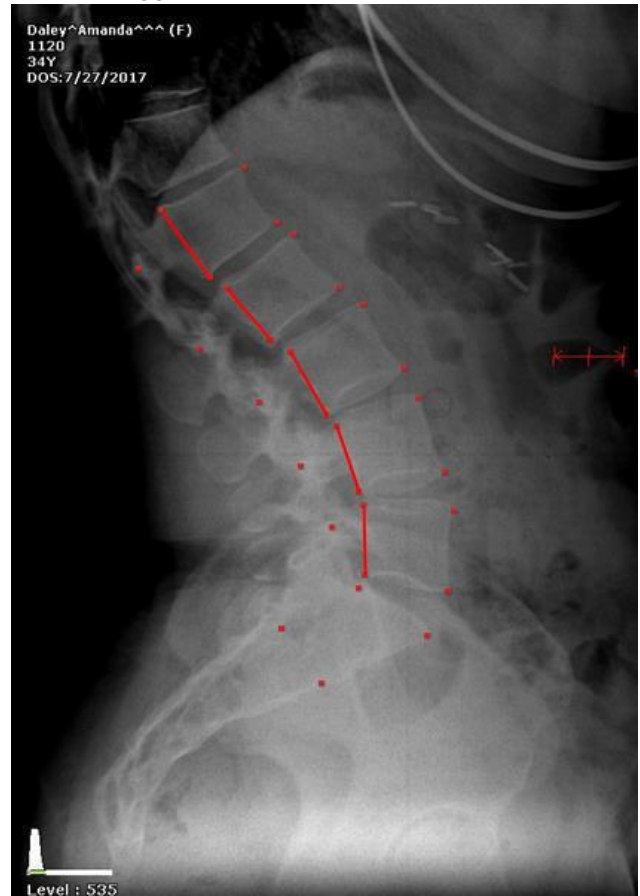


# Lateral Lumbar Spondylolisthesis

STANDARD RESULT



PATIENT RESULT



## Result :

L1-L2 : -8.22 %

L2-L3 : -8.04 %

L3-L4 : -3.69 %

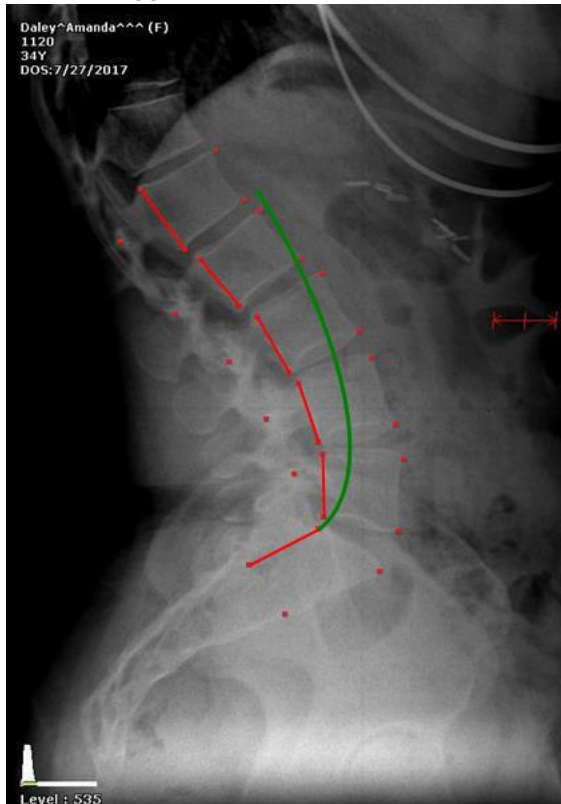
L4-L5 : 0.11 %

L5-S1 : 9.25 %

Spondylolisthesis describes the anterior displacement of a vertebra or the vertebral column in relation to the vertebrae below. The Meyerding grading system categorizes severity based upon measurements on lateral X-ray of the distance from the posterior edge of the superior vertebral body to the posterior edge of the adjacent inferior vertebral body. This is reported based on the total superior vertebral body length. Grade 1 is 0–25%, Grade 2 is 25–50%, Grade 3 is 50–75%, Grade 4 is 75–100%, and over 100% is Spondyloptosis, when the vertebra completely falls off the supporting vertebra.

# Normal Lateral Lumbar Curvature

## PATIENT RESULT



## Result :

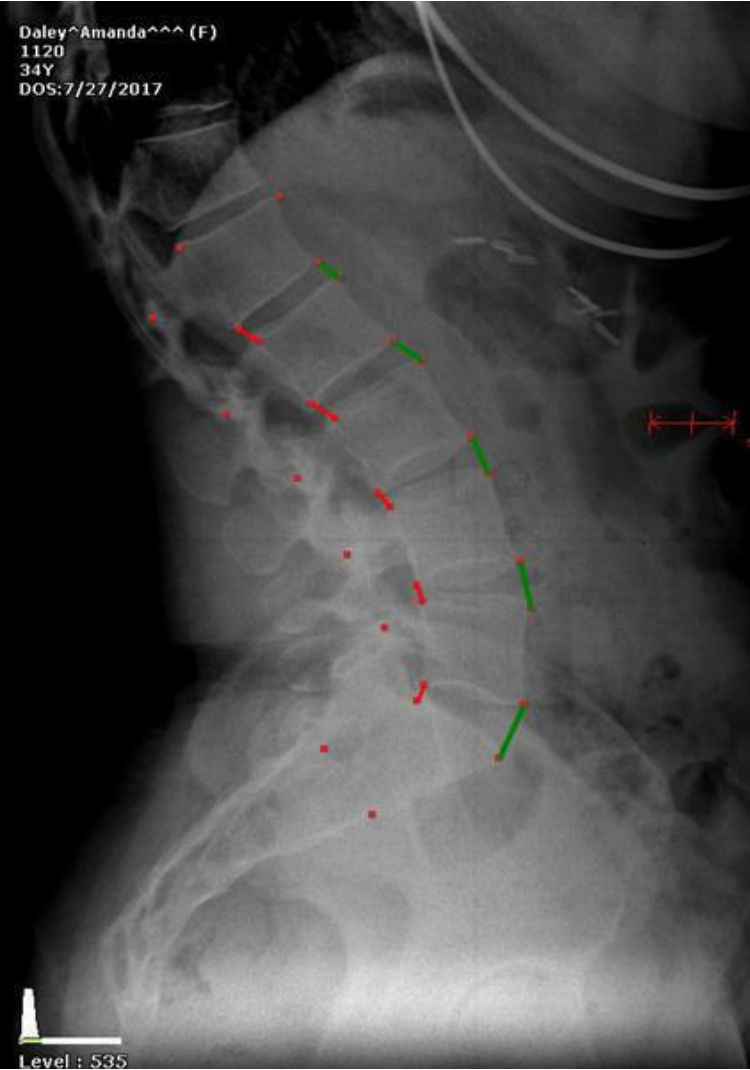
Your posterior translation in your lumbar spine is : 45.33 mm

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The image above shows your vertebral position in red and average or ideal in green. The red lines and the green line should be one on top of the other. Any deviation is generally considered to be abnormal.

# Lateral Lumbar Disc Height

## PATIENT RESULT



### Result :

| Anterior          | Posterior        |
|-------------------|------------------|
| L1-L2 : -5.33 mm  | L1-L2 : -4.67 mm |
| L2-L3 : -7.33 mm  | L2-L3 : -5.33 mm |
| L3-L4 : -13.33 mm | L3-L4 : -5.33 mm |
| L4-L5 : -17.33 mm | L4-L5 : -6 mm    |
| L5-S1 : -20 mm    | L5-S1 : -6 mm    |

The intervertebral disc connects the vertebral bodies to each other and enables complex movement between the individual segments. Degenerative change of the lumbar intervertebral disc has been known to be a cause of back pain. Disc height loss can induce early degeneration change of the facet joint. Disc measurements have a typical pattern of increased height caudally from the 1st and 2nd lumbar vertebrae to the height between the 4th and 5th lumbar vertebrae, and then a slightly decreased height between the 5th lumbar and 1st sacral vertebrae. There has been a tendency in earlier reports to classify this steady biochemical and histologic transformation of the disc as degenerative, but the accounts of Humzah and Soames and and Oda et al. suggest that irreversible changes of disc height are associated with the adaptation of the intervertebral discs to alterations in the prevailing mechanical (functional) conditions within the vertebral column during aging.

# Financial Agreement

Patient Name: **Amanda**

This corrective plan includes all procedures recommended by the doctor during the time period of (7/31/2017 to 10/30/2017)

Any work injury or personal injury (e.g. auto accident) would suspend this plan until that condition was resolved. The plan would then continue and be extended for the unused time purchased.

|                   |                         |
|-------------------|-------------------------|
| \$1,200.00        | 24 visits X \$50 copay  |
| \$52.00           | Cervical Traction Unit  |
| <b>\$1,252.00</b> | <b>Total Investment</b> |

☐ **One Time Payment (Pay in Advance Save 10%)**

|                                             |                    |
|---------------------------------------------|--------------------|
| 10 % Discount for Pay in Advance            | \$125.20 (SAVINGS) |
| Total Payment: (Available next 5 days only) | \$1,126.80 (TOTAL) |

☐ **Monthly Payment**

Total Investment: **\$1,252.00** divided by **4** months: **\$313.00**

Auto Charge: **Visa / MC / Discover Credit Card #:**

Auto Bank Debit:

|                       |                |                |
|-----------------------|----------------|----------------|
| Financial Institution | Routing Number | Account Number |
|-----------------------|----------------|----------------|

☐ **Financing (100% Interest Free Financing Option) Care Credit**

|                 |                              |
|-----------------|------------------------------|
| Monthly Payment | <b>\$69.56</b> for 18 months |
| Approximately   | <b>\$2.32</b> a day          |

*By signing below, I agree to participate in a Chiropractic Corrective Program. If I've selected a payment plan, my credit card or checking account will be automatically charged on the period specified above. To cancel or change this request I must notify Accent on Health Chiropractic in writing and allow a reasonable time to accommodate my request. Should I choose to discontinue my care at any time, I will not be billed for any care not yet received and I will only be responsible for the balance of the care that I have already received. Should I choose to pay in advance for my care, my visits will be calculated on a per visit basis and I will either: a) be responsible for the outstanding balance or b) receive a refund on the care not yet received. None of the payment methods offered guarantee a cure for any condition. ATTENTION: Quoted benefits by your carrier are not a guarantee of payment. Any unpaid services or accumulation towards deductible will be the patient's responsibility.*

7/31/2017

\*Signature of Responsible Party

Date

Staff Signature

# Your Recommended Action Plan

Patient: **Amanda**

Date Prepared: **7/31/2017**

## ***Your Recommended Action Plan is Based On:***

Digital X-ray Findings  
SpineTech Digital X-ray Analysis Shows Increased Bio-Mechanical Stress Loading  
Digital Foot Scans  
Previous Traumatic Injuries  
Digital Posture Analysis Abnormalities  
The Chronic Nature Of Your Problem

*These specific recommendations for your unique problem are based on your individual exam findings and my experience with over 16,000 cases similar to yours over the past 26 years. Healing and pain relief takes time and is not common in a few treatments.*

## ***Doctor Recommended Action Plan:***

Spinal Adjustments  
Extremity Adjustments  
Therapeutic Exercise  
Lowback Kdt Decompression Therapy  
Electrical Stimulation  
Home Cervical (Neck) Compression Traction Unit  
Lumbar ATM2 (Active Therapeutic Motion)  
4 Weeks of Massage Therapy (60 Mins) 1 x Week

## **Your Recommended Treatment Schedule:**

3 times a week for 6 weeks 18 visits

1 times a week for 6 weeks 6 visits

**Total Visits 24 / 3 Months**

## ***Features and Benefits of Your Corrective Care Program, and How It Works:***

Go beyond eliminating pain with postural improvement or correction

Care will be discontinued if you do not show signs of improvement in the first 2 weeks of treatment

David Bohn  
Doctor Signature

Amanda  
Patient Signature

7/31/2017  
Date